



(<https://learningcenter.hfsa.org/>).



My Courses (<https://learningcenter.hfsa.org/Users/Home.aspx>)
/ **2026 AHFTC Board Certification Review - OnDemand**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=4f%2fOexzNsorKAF1lCFotvA%3d%3d>)
/ **2026 AHFTC Board Certification Question Bank**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=YSpCYks4EUa7JCy700Rkkw%3d%3d>)
/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 9 of 117



Which is correct regarding specialized palliative care in patients with advanced HF?

- A. Discussions about code status and goals of care should preferably be initiated by specialized palliative care providers.
-  B. Specialized palliative care consultation improves cost of care and survival in patients with advanced heart failure.
-  C. Specialized palliative care consultation improves quality of life measures, in inpatient and outpatient settings.
- D. Specialized palliative care consultation is reserved for patients who have elected to defer life prolonging therapies.

Commentary

References

Testing Point: The AHFTC specialist should know the effects and indications for specialized palliative care in advanced HF patients.

Answer: C. Specialized palliative care consultation improves quality of life measures, in inpatient and outpatient settings.

Palliative care interventions have shown benefit in quality of life in inpatient and outpatient settings. Palliative care is both a medical specialty (aka Specialized Palliative Care) as well as an overall approach to care that improves quality of life and relieves suffering for patients with serious illness, regardless of prognosis. It differs from hospice in that patients are not required to be at end of life and patients may be on life prolonging therapy. Primary palliative care refers to the routine administration of palliative interventions, such as basic pain management and routine end of life discussions. Primary palliative care can be administered by various care providers such as internists, cardiologists, and advanced heart failure physicians.

Conversely, specialized palliative care providers (those with extra dedicated training in palliation) may be of assistance with complex medical decision making at the end of life, such as contentious transitions to hospice or patients with familial conflicts. Randomized

Interventions in palliative care have shown improvement in quality of life, psychosocial domains and costs but have not been proven to prolong survival in randomized studies. Palliative care consultation is appropriate for advanced heart failure patients even if they are interested in life prolonging care, such as AICDs and stents.

Powered by Oasis.



(<https://www.oasis-lms.com/>).